NCCP 00258 · clinician draft

TCH → trastuzumab continuation

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

TCH · carboplatin Day 1

Active medicines: Docetaxel + Carboplatin + Trastuzumab

Emetogenic risk: High · carboplatin AUC ≥4

Medicine	Dose / route	When
Aprepitant	125 mg orally	Day 1
Ondansetron	16 mg orally	Day 1 Individual dose / QT / hepatic review
Dexamethasone	12 mg orally	Day 1 Reconcile chemotherapy premedication steroids
Aprepitant	80 mg orally daily	Days 2–3
Metoclopramide	10 mg orally up to three times daily	Subsequent days: duration and use to be confirmed Individual contraindications, renal function and duration review
Dexamethasone	8 mg orally daily	Days 2–4 Carboplatin AUC ≥4 antiemetic row; reconcile total steroid exposure

Additional phase-specific support

- Docetaxel: dexamethasone 8 mg orally twice daily for 3 days starting the day before chemotherapy; at least 3 doses before the infusion. If missed, contact the team; any IV replacement is a consultant decision.
- Reconcile the total dexamethasone schedule with antiemetics, including aprepitant interactions. The listed antiemetic steroid is a reference row, not an instruction to add a second course.
- Assess G-CSF according to the exact regimen and individual risk.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00258 v9 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-17 · SACTCheck v0.84.0 · Regimen-level drafts; prescriber and oncology-pharmacy verification required